Hon Meredith Hammat MLA
MINISTER FOR HEALTH; MENTAL HEALTH; PREVENTATIVE HEALTH

In accordance with sections 377 and 378 of the *Mental Health Act 2014*, I submit for your information and presentation to Parliament the Annual Report of the Chief Mental Health Advocate for the financial year ending 30 June 2026.

As well as recording the operations of the Mental Health Advocacy Service for the 2025-26 year, the report reflects on a range of issues that continue to affect people identified under the *Mental Health Act 2014*.

Dr Tony Buti MLA
ATTORNEY GENERAL

In accordance with section 140(2) of the *Criminal Law (Mental Impairment) Act 2023*, I submit for your information and presentation to Parliament the Annual Report of the Chief Mental Health Advocate for the financial year ending 30 June 2026.

As well as recording the Mental Health Advocacy Service's second year of operations under the *Criminal Law (Mental Impairment) Act 2023*, the report reflects on a range of issues affecting identified persons with mental impairment in Western Australia, highlighting key themes and emerging trends in their experiences within the justice system.

Hon Hannah Beazley MLA
MINISTER FOR LOCAL GOVERNMENT; DISABILITY SERVICES; VOLUNTEERING; YOUTH; GASCOYNE

In accordance with section 56(3) of the *Declared Places (Mental Impairment) Act 2015*, I submit for your information and presentation to Parliament the Annual Report of the Chief Mental Health Advocate for the financial year ending 30 June 2026.

The report records the activities of the Advocates who provided services to residents of the Bennett Brook Disability Justice Centre, a declared place, during the 2025-26 year.



Dr Sarah Pollock
CHIEF MENTAL HEALTH ADVOCATE

September 2026

Chief Advocate's foreword

Welcome to the eleventh annual report of the Mental Health Advocacy Service (MHAS). November 2025 marked the tenth year of operation under the *Mental Health Act 2014*. However, rights protection in mental health has a much longer history. The Lunacy Act 1871 obliged 'visitors' to attend every 'asylum' each week and inquire into 'systems of non-coercion', the conditions of 'pauper' patients and the occupation and 'amusements' provided for them.

More recently, the Council of Official Visitors was established under the Mental Health Act 1996 to protect the rights of involuntary patients, whether in hospital or community. Visitors also had an inspection function to identify gaps in mental health services. The *Mental Health Act 2014* further strengthened the rights of and safeguards for people being treated coercively and expanded the functions and powers of Advocates.

Since MHAS' inception in November 2015, demand for advocacy has steadily grown and the number and complexity of the issues Advocates respond to have increased. MHAS' capability in individual and systemic advocacy has developed and our stakeholder relationships in the clinical and non-government service sectors have strengthened. Our *Criminal Law Mental Impairment Act 2023* service continues to evolve and its stakeholder relationships mature.

Of the systemic issues highlighted in the first MHAS annual report, a number are reported again this year: bed pressure and the associated lack of appropriate accommodation and care in the community, access to specialist inpatient treatment for children aged 16-17 years and the conditions in licensed



Dr Sarah Pollock
CHIEF MENTAL
HEALTH ADVOCATE

psychiatric hostels and their oversight and governance. The systemic issues we report on in relation to the *Criminal Law Mental Impairment Act 2023* are similarly predictable. A critical lack of forensic beds, delayed access to assessment reports and lengthy stays in prison without access to comprehensive mental health treatment threaten to undermine the promise of the reforms. And lastly, the excellent Bennett Brook Disability Justice Centre remains woefully under-utilised.

In response to the increased demand and complexity, MHAS received an increase in funding in 2025-26. This will help to address the demand pressures in future years. However, a historical deficit in funding continues to place a strain on service delivery.

I offer my thanks to the people we work with in the various services and agencies. I thank them for their willingness to work with us to achieve better outcomes for identified persons and their families. I know they want to see wider changes as much as we do.

Finally, while Advocates achieve positive outcomes for identified persons every day, it can be disheartening and difficult to maintain momentum when longstanding issues seem to show little sign of change. I thank our Advocates and the public servants who support MHAS, for their commitment and tenacity.

Executive summary

The 2025-26 financial year was one of consolidation, capability building and managing increasing service demand for MHAS.

Following the significant organisational growth and restructuring of the previous year, MHAS focused on embedding its new structure, strengthening governance and workforce capability, improving safe systems of work, and building the relationships required to support sustainable advocacy across Western Australia's mental health, criminal justice and disability justice systems. The year also marked the tenth anniversary of MHAS, providing an opportunity to reflect on a decade of independent advocacy and the continuing importance of safeguarding the rights of identified persons.

Demand for independent advocacy continued to grow. During 2025-26, MHAS supported 5,379 identified persons and Advocates delivered 53,114 hours of advocacy and related work across its three legislative service streams, representing a 16.9 per cent increase from the previous year. In addition to these recorded advocacy hours, Consumer Liaison Officers, Team Leaders and other MHAS workers provided direct support to identified persons, highlighting the broader organisational effort required to deliver advocacy services. The number of issues recorded across all three streams increased by 82.1 per cent to 21,613, reflecting both increasing service demand and the complexity of matters being raised, noting that operational changes also impacted these figures.

Advocacy throughout the year continued to highlight issues relating to safety, access to treatment and care, restrictive practices, procedural fairness, legislative compliance and the experiences of children, young people and other vulnerable groups. Identified persons consistently emphasised the importance of being heard, treated with dignity and respect, and supported to participate in decisions affecting their lives.

Through individual and systemic advocacy, MHAS achieved meaningful outcomes for people and contributed to broader service improvements. Advocacy resulted in amendments to medical records, improved communication with identified persons and families, access to treatment and accommodation, environmental improvements, staff education, formal investigations, and changes to policies, processes and practices. Advocacy also contributed to strengthened incident reporting, greater scrutiny of restrictive practices, improvements in ward conditions, safety management and better access to necessities and support.

The first full year of operation under the *Criminal Law (Mental Impairment) Act 2023* (CLMI Act) provided important insight into how the new framework is working in practice. MHAS received

> **Thank you so much for the care & support you give me.**
>
> **RESIDENT**

519 notifications relating to 343 individuals. Overall, MHAS worked with 406 CLMI identified persons under the CLMI Act during 2025-26, including those who first entered the service in the previous reporting period. Advocates delivered 10,314 hours of advocacy, a 56.8 per cent increase from the previous reporting period. Advocacy activity increased across court proceedings, Mental Impairment Review Tribunal (MIRT) reviews and Community Supervision Orders (CSOs). However, CLMI identified persons continued to experience significant delays in court processes, assessments, access to treatment, accommodation and rehabilitation pathways, as well as challenges navigating multiple health, justice, disability and community service systems.

A major area of systemic advocacy during the year concerned access to mental health treatment for people in prison. Drawing on evidence from individual advocacy, engagement with prisoners and CLMI identified persons, forensic mental health information and stakeholder consultation, MHAS identified significant barriers to timely assessment, treatment and admission for prisoners with serious mental illness. In collaboration with the Office of the Chief Psychiatrist (OCP) and the Office of the Inspector of Custodial Services (OICS) and informed through engagement with the Mental Health Tribunal (MHT), Health and Disability Services Complaints Office (HaDSCO) and other stakeholders, MHAS undertook a major program of systemic advocacy examining prisoner access to mental health care. This work reinforced the need for coordinated action across prison mental health services, forensic and civil hospitals, community treatment, disability supports, accommodation, transition services and cross-agency governance. This advocacy is ongoing.

Organisational capability remained a significant priority throughout the year. MHAS continued to strengthen its workforce, governance, work health and safety, enterprise risk management, data capability and operational systems to support a larger and increasingly complex statutory scope. MHAS also strengthened relationships with Aboriginal Elders and youth, people with lived and living experience, identified persons, community organisations and sector stakeholders. These relationships informed service development, enhanced cultural capability and helped ensure that the perspectives of these communities remained central to MHAS' work.

The growth in service demand was accompanied by significant pressure on the workforce and the organisation's operating model. Advocates and staff worked in increasingly complex and high-risk environments, responding to substantial growth in serious issues, safeguarding matters, distressing communications and multi-agency cases. Recruitment challenges, workforce capacity constraints, exposure to traumatic material and the need for stronger safety, supervision, wellbeing and risk-management arrangements, remain important organisational challenges. MHAS responded through structured training, peer reflective practice, risk assessments, improved escalation pathways and tailored workforce supports.

MHAS recorded expenditure of $10.08 million against a budget of $8.94 million, resulting in an overspend of $1.14 million, or 13 per cent. The variance was primarily associated with salary expenditure, including unfunded positions necessary to meet legislative requirements and salary indexation not automatically incorporated into MHAS funding.

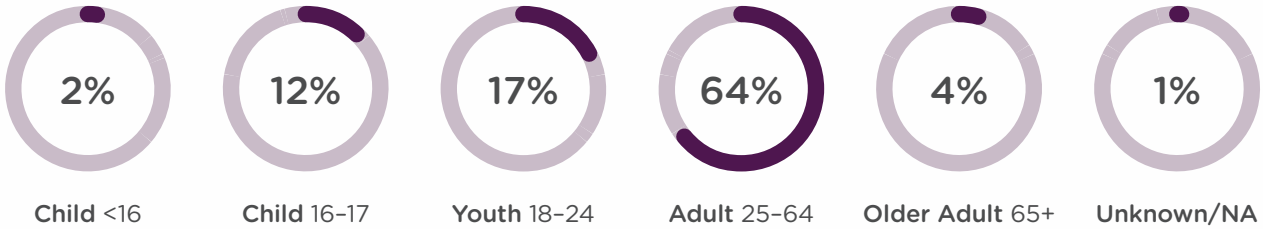
The issues consumers faced

MHAS' role is to support consumers and residents, ensure their voices are heard, protect their rights and assist them to address concerns about their treatment, care and circumstances. The issues reported in this section reflect concerns, experiences and allegations that consumers raised with Advocates, as well as matters identified through advocacy engagement. Advocates take these matters seriously and work with the person and relevant services to understand the concern, obtain relevant information, promote safety and wellbeing, and support an appropriate service response. In many cases, advocacy results in action being taken by services to address concerns, improve communication, provide additional information and explanation, review decisions or implement other changes. Advocates monitor and follow up matters and, where necessary, escalate concerns within the service or through MHAS. The data therefore provides important insight into the experiences of people receiving treatment and care and the issues requiring advocacy intervention. It does not, however, represent formal findings regarding individual allegations or establish the prevalence of issues across the wider mental health system.

As with previous years, consumer and resident issues and complaints data[16] from 2025-26 provided valuable insight into the lived experiences of West Australians navigating the statutory mental health system. The data indicated notable increases across several areas of care and treatment and highlighted a concerning surge in issues and complaints classified by MHAS as 'serious'. MHAS classifies some issues and complaints as 'serious issues' where the matter involves an actual or alleged incident, risk or circumstance that may significantly affect a person's health, safety, wellbeing, dignity or rights. Such issues include allegations of assaults (physical, sexual, verbal, or financial), potential staff misconduct, neglect, or ill-treatment, and suicidality.[17]

In 2025-26, Advocates responded to 683 serious issues representing approximately three per cent of all issues and complaints raised throughout the year. However, the number of serious issues increased by 72 per cent from 397 in 2024-25 to 683 in 2025-26. A greater practice focus on identifying and recording serious issues is likely to have contributed to this increase, although it does not explain the overall scale of the growth. Almost 15 per cent of serious issues were recorded for children under the age of 18 years, while a further 17 per cent were recorded for young people aged 18 to 24 years.

FIGURE SEVEN – A significant proportion of serious issues were recorded for children and youth in 2025-26



> **Thank you again for your incredible support and hard work with my issues at [Hostel]. Your assistance was truly instrumental in giving me the courage and ability to navigate the situation.**
>
> **RESIDENT**

Particularly concerning is that the increase was not isolated to an issue type. Growth occurred across all major categories of serious issues, including physical harm, psychological or verbal abuse, sexual safety concerns, allegations of misconduct, and suicidality. This suggests the increase reflects a broader rise in serious incidents and safeguarding concerns requiring advocacy intervention, rather than a shift in one specific area of risk.

Much of the overall increase in serious issues was attributable to suicidality, which increased by more than 100 recorded occasions. This category includes deaths by suicide, instances where consumers or residents expressed a clear intention to end their lives, and acts undertaken with the intent of dying. Children under 18 accounted for 41 of the 185 suicidality serious issues recorded in 2025-26, or 22 per cent. This included 35 matters involving children aged 16 or 17. While the number of suicidality matters involving children increased, children accounted for a smaller proportion of the category than in 2024-25, when they represented 33 per cent of recorded suicidality serious issues.

Allegations of psychological or verbal abuse also rose considerably, increasing from 19 recorded instances in 2024-25 to 52 in 2025-26 (174 per cent).[18] These reports were raised across a broad range of services and commonly involved allegations of verbal aggression, intimidation and other distressing interactions between consumers and residents in inpatient and residential settings. The increase is concerning. People should be able to access treatment and care in environments where they are treated with dignity and respect. While the increase may indicate a rise in incidents, it may also reflect improved awareness among people of their rights and of the standards of treatment they are entitled to expect, including through engagement with MHAS advocates. Regardless, the number of reports highlights the importance of respectful, rights-based care and the need to address consumer concerns promptly.

[16] Consumer issues and complaints data are coded in the Hub by individual Advocates and may be influenced by variations in recording practices over time. Despite this, the observed trends are considered significant and reflective of increasing demand for MHAS advocacy services.

[17] All serious issue allegations are escalated to a Senior Advocate, who oversees the response to ensure the consumer's immediate safety is prioritised and appropriate action is taken by the relevant service or individual.

[18] Significant increases were recorded across several issue categories in 2025-26. These are likely to reflect a combination of factors, including greater advocacy activity and engagement with consumers or residents; increased consumer and resident awareness of their rights and the standards of treatment and care they are entitled to expect; and changes in the volume, nature and complexity of matters raised with advocates. Continued development of the Hub has also supported more consistent and detailed recording of issues. The figures should therefore be understood as measures of issues identified and recorded through MHAS advocacy. An increase in recorded issues reflects a combination of increased identification and reporting, and a genuine increase in issues experienced.

There was also a substantial increase in allegations of physical abuse, rising by 54 per cent, from 68 to 105 recorded allegations. Consumers and residents reported increasing concerns about peer aggression and violence within inpatient units and psychiatric hostels, often occurring in the context of rising acuity, behavioural disturbance and, in some settings, alcohol and other drug (AOD) use. While inpatient services have increasingly relied on security officers and nursing specials to manage risk, physical assaults and aggression between consumers or residents remain a significant concern including in licensed private psychiatric hostels.

Sexual safety issues also increased by 41 per cent to 97 recorded occasions. These matters ranged from allegations of sexual assault to sexually disinhibited behaviour, inappropriate sexual conduct and concerns about the vulnerability of consumers within treatment and care environments. Combined with the reported increase in physical and psychological harm, these figures raise broader concerns about the ability of some treatment environments to consistently provide safe, therapeutic and recovery-oriented care.

The substantial increase in serious issues recorded during 2025-26 raises significant concerns about the safety and quality of care being experienced by some people within mental health services. While MHAS does not determine whether allegations are substantiated, it takes each allegation at face value and works to ensure concerns are promptly escalated, investigated and responded to by the relevant service. Each report requires careful assessment and often intensive advocacy engagement to support the person and advocate for appropriate action. The growth in serious issues, particularly those involving suicidality, physical harm and sexual safety, highlights the need for continued focus on safeguarding within mental health services. People should be able to access treatment and care in environments that are safe, trauma-informed and recovery-focused, and the trends observed during 2025-26 suggest there is more work to be done to ensure services consistently promote safety, dignity, wellbeing and recovery.

TABLE TWO – The increasing number of 'serious issues' recorded by Advocates is concerning

	2023-24	2024-25	2025-26	Change
Alleged physical abuse	72	68	105	▲ 54.4%
Alleged psychological or verbal abuse	15	19	52	▲ 173.7%
Sexual safety	82	69	97	▲ 40.6%
Alleged financial abuse	6	7	8	▲ 14.3%
Potential misconduct, wilful neglect or ill treatment	106	87	101	▲ 16.1%
Other potential serious issues	48	66	135	▲ 104.5%
Suicidality	14	81	185	▲ 128.4%
Total	343	397	683	▲ 72.0%

Advocacy in action: systemic issues and outcomes

The concerns raised with Advocates during 2025-26 provide a unique perspective on the experiences of consumers and residents receiving treatment, care and support across Western Australia's mental health system. Advocacy identified recurring issues relating to safety, access to treatment and care, restrictive practices, procedural fairness, and legislative compliance. During the year, MHAS recorded 20,704 issues raised by consumers and residents subject to, or receiving services under, the MH Act, of which 683 were categorised as serious.

These issues reflect concerns, experiences and allegations raised through advocacy, as well as matters identified by Advocates through their engagement with consumers, residents, and services. Advocates work with consumers and residents, seek information from services, monitor outcomes and, where necessary, escalate concerns. Many issues result in action, clarification, review, or other responses by services. Consequently, the information presented in this section provides insight into the issues requiring advocacy intervention and the recurring themes identified through advocacy practice. Consumers and residents consistently emphasised the importance of being heard, treated with dignity and respect, and supported to participate in decisions affecting their lives.

Safety

Safety concerns reported by consumers and residents almost doubled during the year, increasing from 291 issues in 2024-25 to 569 issues in 2025-26 (as shown in Figure Eight). Table Two records 683 serious issues, including 105 alleged physical abuse matters, 52 alleged psychological or verbal abuse matters, 97 sexual safety matters, 101 matters involving potential misconduct, wilful neglect or ill-treatment, and 185 involving suicidality.

Safety concerns ranged from allegations of physical assault and sexual safety incidents to concerns about staff conduct, suicidality and responses to serious incidents. People reported feeling unsafe because of aggression, violence or intimidation from other people receiving services. In youth services, suicidality, significant self-harm, physical harm and sexual safety remained recurring concerns, often involving young people with complex needs and requiring sustained advocacy involvement.

The adequacy of service responses following incidents was a recurring concern. Some people reported that, despite incidents having occurred, they did not feel the actions taken by services adequately addressed ongoing safety risks or restored their sense of safety. Some people described the impact of being assaulted, threatened or exposed to aggressive and volatile treatment environments, including the effect of witnessing violence, threats or restrictive interventions on their sense of safety, wellbeing and recovery. Concerns also arose where incidents were not documented or escalated, families received limited information about what had occurred, or actions taken by services did not appear proportionate to the identified risks.

Advocates reviewed records and closed-circuit television footage, supported people and families to participate in investigations, raised complaints and inquiries, and escalated unresolved concerns. Outcomes included formal investigations, retrospective documentation and notification of incidents, amendments to records, apologies, staff education, service reviews, environmental and security improvements, and changes to local practice and policies.

Despite these outcomes, the scale of growth in both consumer – or resident – reported safety concerns and serious issues, indicates that safety remained a significant concern across mental health services this year.

Use of restrictive practices

Concerns about restrictive practices and restrictions on communication have also increased.

Table Ten shows that concerns relating to restrictive practices and communication increased in 2025-26. Issues concerning seclusion increased from 30 matters in 2024-25 to 38 in 2025-26, restraint increased from 78 to 95, and issues relating to access to communication increased from 145 to 282. Separate Hub notification data recorded 630 Form 12C notifications concerning restrictions on freedom of communication, relating to 356 unique consumers.

TABLE TEN – Concerns relating to restrictive practices increased in 2025-26

No. of occasions	2024-25	2025-26	Change
Seclusion	30	38	▲ 26.7%
Restraint	78	95	▲ 21.8%
Access to communication	145	282	▲ 94.5%

Consumers described the distress associated with seclusion, restraint, sedation and communication restrictions. Advocacy concerns included prolonged or repeated restrictive practices; access to toilets, water, clothing and communication during seclusion events; the quality and frequency of monitoring and observation; the involvement of security staff; and whether less restrictive alternatives had been considered. Advocates also raised concerns regarding the use of chemical sedation and highlighted cases where consumers experienced serious physical health consequences, including requiring admission to medical wards or intensive care, before being able to access appropriate mental health treatment. Some incidents involving restrictive interventions resulted in significant injuries to consumers and resulted in complaints, inquiries, and reviews of practice.

For children and young people, concerns also included removal of clothing or underwear, access to bedrooms, leave and communication restrictions and restrictive responses to self-harm, with implications for privacy, dignity and developmentally appropriate care.

Advocates reviewed clinical records and CCTV footage, attended seclusion and restraint review meetings, supported complaints and inquiries, and escalated concerns. Outcomes included investigations, completion of mandatory notifications, amendments to records, staff education, reviews of restrictive-practice processes, and improved communication with consumers and families.

Coordination for people with complex and intersecting needs

Advocates worked with consumers and residents whose needs crossed mental health, physical health, AOD, disability, housing, child protection and justice systems. Advocacy work identified fragmented service delivery, delays and uncertainty about agency responsibility where multiple systems were involved. In practice, this contributed to incidences of repeated emergency presentations, prolonged admission, delayed discharge, placement breakdown, accommodation barriers, and gaps in sustained community support.

These challenges were particularly evident where mental illness intersected with disability, child protection involvement, homelessness risk or justice-system contact. Consumers, residents and families often described difficulty identifying which agency was responsible for responding to their needs, while services sometimes disagreed about roles, funding responsibilities or appropriate support pathways. As a result, consumers could remain in hospital longer than clinically necessary or experience delays accessing accommodation, disability supports or community services.

Advocates brought services together, clarified responsibilities, ensured consumers' views were heard and supported more coordinated responses. In several youth matters, persistent advocacy contributed to multi-agency responses to secure assessments, accommodation, funding, and support packages. Advocacy also assisted consumers and families to participate in decision-making and escalate barriers that could not be resolved at a local service level.

Experiences of hostel residents

Licensed private psychiatric hostels accommodate more than 700 residents across Western Australia and provide accommodation and support for people living with significant and ongoing mental health needs. MHAS reports separately on hostel advocacy because of the different regulatory and operating context from advocacy in authorised mental health units. Hostel residents' concerns frequently extend beyond mental health treatment and involve accommodation security, provider practices, daily living conditions, poor personal care support, access to clinical and AOD services, financial hardship and the risk of homelessness if a placement breaks down.

AOD use continued to affect resident safety, mental health and accommodation stability. Advocates identified matters involving aggression, conflict, property damage, warnings and threatened or actual eviction. Residents wanting to access detoxification or rehabilitation could experience difficulty managing their hostel accommodation charges while also meeting the cost of treatment. Advocates linked residents with treatment and harm-reduction services, engaged with providers about warnings and eviction notices, sought additional time for residents to address concerns and advocated for alternative accommodation where placements could not be maintained.

Safety, dignity and respect were also recurring concerns. Residents reported feeling unsafe where providers were unable to manage aggression or conflict involving other residents, particularly where mental health deterioration or AOD use was involved. Some residents considered that safety issues were not taken seriously or were responded to inappropriately. Residents also raised concerns about disrespectful treatment, limited involvement in decisions affecting their lives, privacy and sexual safety, and practices that reduced their autonomy within their living environment.

Residents experienced difficulties obtaining appropriate responses when their mental health deteriorated, including barriers to re-engaging with community mental health services. These difficulties were particularly evident for residents who had been discharged to general practitioner-led care and later experienced deterioration. Staffing levels, high turnover,



Hostels before and after advocacy.

reliance on inexperienced staff and inconsistent responses to mental health deterioration, conflict and crisis affected the continuity and quality of support. These concerns were particularly evident outside ordinary business hours and in larger congregate living environments.

The principal accommodation and daily living concerns included:

- poor food quality, insufficient quantity and limited choice
- ageing infrastructure and delayed maintenance
- inconsistent cleaning and compliance with accommodation standards
- inadequate heating and cooling
- deteriorated furniture, mattresses and other household items
- difficulty securing rooms or obtaining replacement keys
- delays accessing clothing, toiletries and other essential items
- accommodation-related charges affecting residents experiencing financial hardship

These matters affected residents' comfort, safety, dignity, autonomy and quality of life. Older residents, residents with physical or mobility limitations, residents without regular family or guardian oversight and residents dependent on providers for essential household items and environmental supports were particularly affected by some of these concerns.

Advocates engaged directly with hostel licensees, raised complaints and inquiries, escalated licensing concerns and advocated with clinical, housing and other services. Outcomes included the withdrawal or extension of eviction notices, prevention of homelessness, alternative accommodation, access to Department of Communities (Communities) housing, repairs and security improvements, furniture replacement, improved cleaning and food arrangements, access to clothing, toiletries and other essential items, access to medical appointments, financial redress, apologies, reinstatement of resident meetings, improved family contact and stronger communication with providers. Some significant environmental and service concerns remained unresolved despite ongoing advocacy.

Accountability for hostels is shared between the Commission, through funding and contractual arrangements; the Change to Department of Health (DoH) Licensing and Regulatory Unit, through licensing standards; the OCP, through the Chief Psychiatrist's standards for clinical care; and MHAS, in relation to protecting residents' rights. Pathways for resolving issues varied in effectiveness. Some providers responded constructively, while other matters required escalation to one or more agencies with relevant oversight responsibilities and did not always result in timely or effective resolution. The persistence of some issues indicates gaps in the collective accountability these agencies share for the care, support, safety and living conditions experienced by hostel residents.

Who we worked with and the issues they raised

In 2025-26, MHAS received 519 notifications relating to 343 unique individuals across the notification categories reported in Table Eleven, compared with 391 notifications relating to 326 unique individuals during the previous reporting period. Given that 2024-25 captured only the first ten months of operation following commencement of the reforms on 1 September 2024, an increase in overall notifications was anticipated.

These figures should not be interpreted as the total number of CLMI identified persons assisted by MHAS during the year. In 2025-26 MHAS worked with 406 people, including those who first came into contact with the service in a previous reporting period.

The most common notification type continued to occur under section 30 of the CLMI Act, where a person's fitness to stand trial was raised during a court proceeding. Notifications of this type increased by 11 per cent, while the number of unique individuals increased by five per cent. This growth suggests courts and legal representatives are becoming increasingly familiar with the legislation and more willing to utilise the pathways available under the CLMI Act.

The notification data should, however, be interpreted with caution. Throughout the year, MHAS continued to identify issues regarding the timeliness, completeness and accuracy of court notifications. MHAS and Courts and Tribunal Services remained engaged in ongoing efforts to improve notification processes, including regular communication, escalation of issues and process improvements. While these efforts supported some improvements, MHAS continued to identify matters that were not notified by the court and instances where notifications did not include information such as notification dates or relevant hearing dates. As a result, reported notification figures are likely to understate total CLMI activity and do not fully reflect how promptly people were identified and connected with the statutory advocacy pathway.

Notwithstanding these limitations, the pattern of notifications provides important insight into how the reforms are evolving. While fitness to stand trial proceedings remained the largest source of referrals, growth in review-related notifications suggests increasing numbers of CLMI identified persons are progressing beyond initial court proceedings into ongoing supervision and review processes established under the CLMI Act. Together, these trends suggest demand is becoming increasingly distributed across multiple stages of the criminal justice pathway.

Custody Orders and Community Supervision Orders

One of the most notable developments during the year was the significant increase in matters involving the MIRT. The MIRT oversees CSOs and COs made by the court to ensure decisions are regularly reviewed and appropriately based on the person's risk and rehabilitation potential. Notifications relating to review proceedings increased from 50 to 209, while the number of unique individuals almost doubled from 39 to 72. This growth reflects the increasing number of people progressing to the review and oversight mechanisms established by the CLMI Act.

Similarly, notifications regarding orders made under section 48 (court makes a CO or CSO) increased during the year. Six new COs were made, compared with four during the previous reporting period, while there was a sizeable increase in the number of CSOs, rising from two to 21. Only one of these CSOs was issued for a child. Although these numbers remain relatively small, and should be interpreted with caution, they represent an important shift in the operation of the reforms. CSOs did not exist under the previous legislative framework and are intended to provide a less restrictive pathway for suitable individuals while maintaining community safety.

TABLE ELEVEN – CLMI related notifications increased in the past year, largely via MIRT review proceedings

	2024-25		2025-26	
Notification type	Notifications	Individuals	Notifications	Individuals
Fitness to stand trial raised (s30)	236	209	259	220
Transitional cohort	79	54	NA	NA
MIRT review proceedings	50	39	209	72
Notification of place of custody (s63)	5	3	5	5
Adjournment to see if become fit (s36)	5	5	5	5
Notification of order (s48) – Custody Order	4	4	6	6
Notification of order (s48) – Community Supervision Order	2	2	21	21
Custody order discharged	3	3	2	2
Other miscellaneous	7	7	12	12
Total	**391**	**326**	**519**	**343**

TABLE TWELVE – There was some minor variability to the place of custody for existing Custody Orders

		1 Sep 2024 – 30 Jun 2025	2025-26
Number of notices received under Reg 3 Under CLMI Act and s48 of CLMI for Custody Orders made by the court		4	6
Total Custody Orders in place where the place of custody was	Authorised Hospital	33	38
	Declared Place	3	3
	Prison	8	4
	Total	**44**	**45**

The increase in review proceedings and CSOs suggests advocacy needs are changing as the reforms begin to mature. Increasingly, Advocates are supporting people not only through fitness proceedings but also through review hearings, rehabilitation planning, supervision arrangements and progression towards less restrictive options.

The total number of COs in place at any time during 2025-26 remained broadly consistent with the previous year. Information provided by the MIRT indicated a shift in the recorded place of custody, with the number of COs involving a prison falling from eight to four and the number involving an authorised hospital increasing by five. This suggests that a greater proportion of consumers subject to a CO were being managed within hospital settings rather than prison settings during 2025-26.

MHAS considers it preferable for people with mental illness who are subject to the CLMI Act to receive treatment and support in therapeutic environments rather than prison. People subject to the CLMI Act who remain in prison may be particularly vulnerable, and custodial environments are not designed to provide therapeutic care. Limited access to mental health assessment, treatment and psychiatric services can adversely affect wellbeing and recovery.

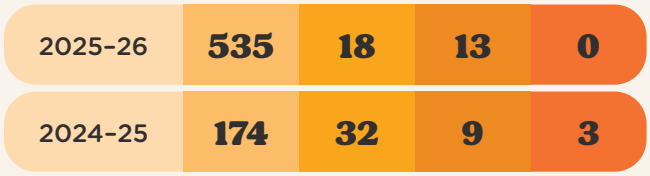
> I affirmed [Advocate] for being so proactive in approaching me as many families find this process completely confusing and not even be aware of CLMI. Often the court process is overwhelming, the lawyer is appearing for the individual and rushing off to other cases. So, her proactive and informed approach was what we need more of in courts!
>
> **PARENT OF CLMI IDENTIFIED PERSON**

Children aged 0 to 17 years

Children represented approximately 11 per cent of section 30 CLMI notifications received during 2025-26. MHAS received 29 notifications relating to 23 unique children, six of whom were recorded as being First Nations. All notifications were received under section 30 of the CLMI Act, where the child's fitness to stand trial was raised. Of the 29 notifications, more than half (16) were for children younger than 16 years of age.

As the CLMI legislation has become more embedded, the work of Advocates has increased in frequency and complexity. The time Advocates spent on individual tasks more than doubled compared to the previous reporting period (increasing from 2,462 hours to 5,769 hours). For children, this was even more stark with Advocate task hours increasing by 155 per cent. Like last year, Advocates spent most of their time on tasks related to processes involved in determining the fitness of the accused.

FIGURE FOURTEEN – Advocate task hours supporting CLMI identified children more than doubled over the last year



- Unfit Accused
- Community Supervision Order
- Hospital Order
- Custody Order

Delays, complexity and navigating the CLMI pathway

The most consistent theme identified through advocacy during 2025-26 was delay. Delays were evident across multiple stages of the CLMI pathway, including court proceedings, fitness assessments, psychiatric and psychological reporting, access to treatment, admissions to the Frankland Centre, review hearings and transition planning. Advocates regularly encountered matters where expert reports took many months to complete, hearings were repeatedly adjourned, or CLMI identified persons remained in custody awaiting treatment, accommodation or support services.

For many people, these delays had consequences extending well beyond the legal process. Court proceedings were often described as slow, confusing and difficult to understand, particularly where matters were repeatedly adjourned, or progress depended on specialist assessments and reports. Many people reported feeling powerless, excluded from decision-making and uncertain about what needed to occur before their matter could progress. Advocates played a key role in ensuring procedural fairness by helping CLMI identified persons understand the process, obtaining information about the status of their matters, communicating with legal representatives and agencies and participating more effectively in decisions affecting their lives.

Advocates observed that prolonged delays could have serious consequences for wellbeing, recovery and progression through the CLMI pathway. Extended periods of uncertainty, detention and untreated needs often contributed to increasing distress, frustration and hopelessness. In custodial settings, Advocates frequently encountered people who had been waiting for extended periods for psychiatric review, hospital admission or access to treatment while their legal matters remained unresolved. Delays in assessment and treatment were associated with increased risks of self-harm, suicidality, worsening psychosis or other symptoms and behavioural escalation. In some matters, Advocates observed that unmet mental health needs appeared to contribute to further offending behaviour or incidents within custodial settings, highlighting the importance of timely access to assessment, treatment and therapeutic intervention. Where concerns arose, Advocates worked with services, legal representatives and families to address barriers, escalate concerns and progress access to assessment, treatment, accommodation and support services. In some matters, this resulted in improved access to assessments, treatment planning and pathways to less restrictive arrangements.

A particular challenge was the availability of specialist fitness assessments. Limited clinician availability, combined with increasing demand, contributed to lengthy waiting periods for reports required by the courts. In some matters, individuals remained subject to detention and ongoing legal proceedings for several months while awaiting assessments necessary for decisions about fitness, treatment or progression through the CLMI pathway. Advocates assisted CLMI identified persons and their families to understand the assessment process, monitored the progress of matters, sought information regarding delays and anticipated timeframes, and regularly followed up with relevant agencies to advocate for timely completion of assessments and reports.

The first full year of operation highlighted the complex relationship between legislative safeguards and practical outcomes. While the CLMI framework provides important protections and review mechanisms, the experiences of CLMI identified persons demonstrated that delays in assessments, hearings, treatment pathways and accommodation options can significantly reduce the practical benefits of those safeguards. This was particularly evident where suitable treatment, rehabilitation or community-based alternatives were unavailable, resulting in people remaining in restrictive environments longer than necessary. Advocates regularly worked with agencies, legal representatives and support providers to explore less restrictive options, promote access to treatment and supports, and facilitate planning for transitions where opportunities became available. However, these efforts were often constrained by broader system capacity, workforce shortages and the availability of suitable services and accommodation.

Statewide forensic bed capacity

While government has commenced a major program of work to expand forensic mental health capacity, including through the Graylands Reconfiguration Proejct and subsequent expansion of the Frankland Centre, demand for forensic mental health services continued to exceed available capacity during 2025-26. Advocacy work undertaken through MHAS' CLMI service stream and broader prisoner access to mental health project also continued to identify substantial barriers to timely access to treatment.

As at 30 June 2026, the Frankland Centre comprised 30 beds, while Dryandra Ward provided a further 14 forensic rehabilitation beds available only to prisoners on COs (13 operational at 30 June 2026). Together, these 43 operational beds served a prison population of approximately 8,700 people.

Access to forensic mental health services became a significant advocacy issue during the reporting period. Through individual advocacy, engagement with people subject to the CLMI Act, prison visits and MHAS' systemic work examining prisoner access to mental health treatment, Advocates continued to encounter matters where access to hospital-based care was delayed or difficult to obtain.

MHAS' quarterly reporting data indicated that 20 of the Frankland Centre's 30 beds were occupied by CLMI identified persons on COs. Of the remaining 10 beds, five were occupied by long-stay prisoners receiving treatment under the MH Act. As a result only five beds were available for other prison referrals. As at 30 June 2026, Acacia Ward, which provides treatment for people requiring the highest levels of care, was operating at near-full capacity, with seven of its eight beds occupied and the remaining bed reserved for a person on a CO awaiting transfer from prison. Dryandra Ward was similarly operating at near-full capacity, with all 13 of its operational beds occupied.

Collectively, these capacity pressures limited the availability of beds for prisoners requiring admission to forensic mental health services.

Furthermore, in the first six months of 2026 there was an average of 22 prisoners referred for treatment to the Frankland Centre.[24] For CLMI identified persons, this could mean remaining in custodial environments for extended periods of weeks or months while awaiting access to treatment services more appropriately delivered in a therapeutic setting.

Access to inpatient treatment was not solely affected by bed availability. Advocates observed that access to hospital-based mental health care could also be influenced by the coordination required between health and justice agencies, differing views regarding the most appropriate treatment setting and uncertainty regarding available admission pathways. While mechanisms existed to facilitate assessment, referral and coordination, including through SHOC and other established liaison arrangements, Advocates continued to encounter matters where treatment options, admission pathways and agency responsibilities required clarification before access to care could be progressed. These experiences suggest opportunities for greater consistency, clearer protocols and improved coordination between health and justice agencies in supporting access to mental health treatment for prisoners. MHAS acknowledges ongoing work across agencies to strengthen these arrangements.

The systemic advocacy work undertaken by MHAS regarding prisoner access to mental health care reinforced concerns identified through individual advocacy. While advocacy contributed to improved access to assessment, treatment and admission in some matters, these outcomes were often difficult to achieve and did not resolve the underlying barriers affecting access to care. Progress remained constrained by bed availability, workforce shortages, competing service demand and uncertainty regarding treatment pathways. The experiences of CLMI identified persons throughout the year reinforced the need for improved access to treatment, clearer pathways, a shared understanding of roles and responsibilities, and coordinated responses across health, justice and disability systems.

What MHAS did

MHAS undertook a structured body of work that combined advocacy evidence, policy analysis, consultation and report drafting. This included drawing together information from individual advocacy matters, prison engagement, CLMI work, forensic mental health data, oversight material, and stakeholder discussions. This work:

- Analysed advocacy trends and recurring issues identified through individual cases involving prisoners and CLMI identified persons
- Examined pathways into forensic mental health services, including access to the Frankland Centre and barriers to hospital admission
- Reviewed available referral, admission and service capacity information to better understand the difference between recorded demand and likely unmet need
- Consulted with SFMHS, Corrective Services, OICS, OCP, MHT and HaDSCO and other stakeholders
- Considered access to advocacy for prisoners referred for mental health assessment and treatment, including practical issues associated with Form 1A pathways
- Developed briefing material, analysis and reform options to support discussion with Government, senior decision-makers and mental health stakeholders

What MHAS produced

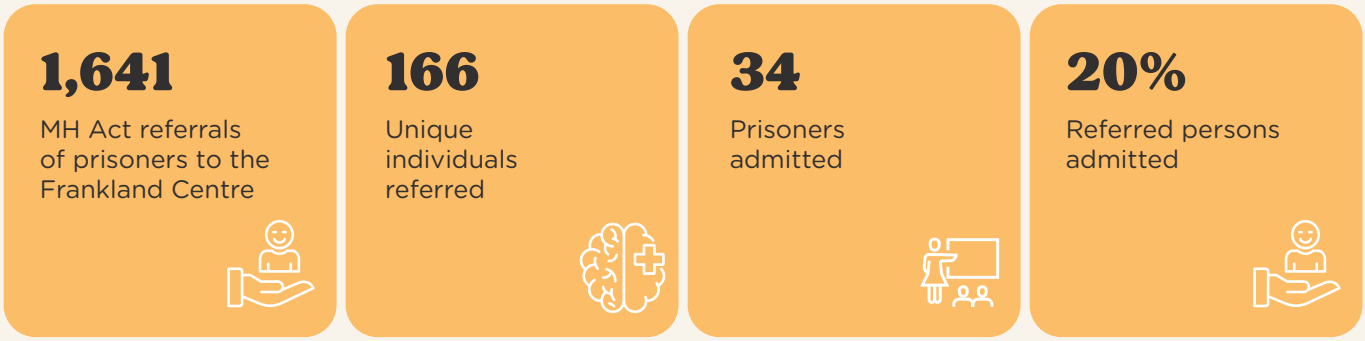
The work produced a substantial evidence-informed briefing paper on prisoners' access to mental health care and treatment in Western Australia. It also generated internal briefing material, presentation content, advice for executive-level discussions, analysis of referral pathways, and sharper articulation of the links between prison mental health access, forensic bed pressure, CLMI implementation, disability pathways and broader system reform.

What MHAS achieved

Through this work, MHAS elevated prisoner access to mental health care as a significant systemic issue. It provided a clearer evidence base for reform discussions and helped shift the conversation from a narrow focus on forensic beds to a broader whole-of-system response.

MHAS advocacy emphasised that forensic beds are critical, but they are only one part of a wider continuum of care. Reform also requires effective prison mental health services, timely assessment pathways, civil hospital and emergency pathways where appropriate, community treatment and support, disability justice pathways, supported accommodation, step-down options and stronger cross-agency governance.

FIGURE EIGHTEEN: What the evidence showed for prisoners in WA[25]



1. Demand for forensic mental health services exceeds available capacity

Service data and stakeholder consultation consistently indicated that demand for forensic mental health assessment and treatment exceeds available capacity. Capacity constraints were identified as a key barrier to timely access to hospital-level care.

2. Data underestimates unmet need

MHAS identified concerns that referral and admission data may not fully reflect demand for forensic mental health services. Consultation suggested that prisoners can remain awaiting bed availability without progressing through ongoing referral pathways, reducing the visibility of unmet need within reported data.

3. Prisoners often experience extended periods awaiting access to treatment

The work identified concerns that prisoners requiring specialist mental health treatment may experience significant delays before hospital admission occurs. This can prolong time spent in custodial environments that are not designed to provide hospital-level care.

4. Forensic beds are only one component of a broader treatment pathway

While forensic bed availability emerged as a significant issue, MHAS found that access to treatment is also affected by prison mental health services, admission pathways, discharge arrangements and community supports. Increasing bed capacity alone is unlikely to resolve broader system pressures. There is also an assumption that all prisoners require a forensic bed, whereas many prisoners' treatment in a civil hospital would be appropriate for their inpatient care.

5. Prison mental health reform requires a system-wide response

The work concluded that improving access to mental health treatment requires coordinated action across health, justice, and related service systems. Effective reform depends on strengthening pathways between assessment, treatment, hospital care and ongoing supports.

[25] Source note: SFMHS data provided to MHAS for the year from 1 September 2024 to 31 August 2025 reported 1,641 MHA referrals of prisoners to the Frankland Centre for 166 unique individuals, with 34 admissions. The same advice reported an average Frankland Centre occupancy of 99% in the current financial year and noted that new admissions could only occur into the acute eight-bed Acacia ward.

The residents of the DJC

Like previous years, the number of people residing at the DJC continued to remain low in 2025-26. Again, only three people were accommodated at the facility throughout the year and only one new MIRT determination directed that a person subject to a CO be placed at the DJC. All three residents were First Nations people. In contrast, 11 new determinations were made to place people on new COs at authorised hospitals, increasing from five the year before. At the end of the reporting period, only three of the DJC's 10 beds were occupied, continuing a pattern of low utilisation observed in previous years, as there has never been any more than three residents at any time since the centre opened.

The custody data also indicate that placement patterns remained relatively stable despite the increased number of new MIRT determinations in 2025-26. The total number of COs where the place of custody was the DJC remained unchanged at three, while the number where the place of custody was an authorised hospital increased from 33 to 38. Although 13 new MIRT determinations were made during the year, the total number of people on COs increased by only one. This reflects that new determinations are only one factor affecting the total number of COs in place at any point in time, with the figures also influenced by people exiting orders or changes in place of custody during the reporting period.

MHAS Advocates received seven initiations through MIRT Review Proceedings during the year involving three DJC residents. Advocates completed 70 advocacy tasks[26] across the three residents which included contacting each resident at least four times throughout the year as a minimum requirement. Residents are not required to engage with advocacy services and may decline advocacy contact at any time. However, all residents engaged with MHAS during the reporting period.

TABLE THIRTEEN – The number of new determinations at Authorised Hospitals doubled last year

		1 Sep 2024 – 30 Jun 2025	2025-26
Number of notices received under Reg 3 Under CLMI Act and s48 of CLMI for Custody Orders made by the court		4	6
Total Custody Orders in place where the place of custody was	Authorised Hospital	33	38
	Declared Place	3	3
	Prison	8	4
	Total	**44**	**45**
Total new determinations made by the Tribunal for place of custody to be	Authorised Hospital	5	11
	Declared Place	1	1
	Prison	3	1
	Total	**9**	**13**

[26] Advocacy tasks are individual actions undertaken by an Advocate on behalf of a resident. Tasks may include contact with the resident, provision of rights information and advice, preparation for MIRT proceedings, attendance at hearings, liaison with services, follow-up work and other advocacy activities recorded in the Hub.

Under-utilisation of the DJC

The role of the DJC within the broader forensic disability, mental health and justice systems remained an area of discussion during 2025-26. While the DJC is intended to provide a secure, therapeutic and residential alternative to custody for people subject to the DPMI Act, Advocates and legal stakeholders continued to raise questions about the facility's limited utilisation.

Concerns focused on whether the current approach is sufficiently responsive to people with complex or intersecting needs, including neurodivergent people and those whose circumstances do not align neatly with more traditional disability categories, especially those who had not been assessed for disability during childhood. MHAS observed a lack of clarity regarding decision-making responsibility for placement at the DJC. While the MIRT advised that it relies heavily on the eligibility and suitability assessments provided by Disability Justice Services, Disability Justice Services noted that the final determination regarding placement rests with the MIRT. This created uncertainty for identified persons, legal representatives and Advocates seeking to understand the basis of placement decisions or avenues for review.

Advocates observed that there is currently no clearly defined pathway for reconsideration of placement where a person is initially assessed as unsuitable for admission to the DJC. This may create barriers for individuals whose circumstances have changed (for instance, effective symptom management through treatment for mental illness) or whose support needs could potentially be met through a different service response at a later point in time. Greater clarity regarding reassessment pathways may assist courts, legal representatives and decision-makers to identify appropriate alternatives and ensure placement options remain responsive to changing circumstances. During the 2025-26 year MHAS was encouraged by advice from the Disability Justice Service that the eligibility criteria are intended to be reviewed during 2026-27.

MHAS also recognises that the suitability of the DJC for some residents is influenced by the availability of timely mental health and physical health support pathways when a resident's condition deteriorates. Access to specialist assessment, treatment and inpatient care can be an important component of managing complex needs within a disability justice setting. Strengthening these pathways may enhance the DJC's capacity to safely support residents with complex and intersecting needs and increase confidence in considering referrals for people who may previously have been assessed as requiring a higher level of health system support.

Greater transparency regarding assessment processes, eligibility criteria and placement decisions would assist identified persons, legal representatives and decision-makers to better understand the role of the DJC within Western Australia's forensic disability system. It may also support more consistent pathways for people whose needs sit at the intersection of disability, mental health and justice services.